



28 November 2006

Statement of Issues - Johnson & Johnson's proposed acquisition of the consumer healthcare business of Pfizer Inc

1. Outlined below is the Statement of Issues released by the Australian Competition and Consumer Commission (ACCC) in relation to the proposed acquisition of the consumer healthcare business of Pfizer Inc (**Pfizer**) by Johnson & Johnson (**J&J**) (**the proposed acquisition**).
2. A Statement of Issues published by the ACCC is not a final decision about a proposed acquisition, but provides the ACCC's preliminary views, drawing attention to particular issues of varying degrees of competition concern, as well as identifying the lines of further inquiry that the ACCC wishes to undertake.
3. In line with the ACCC's *Merger Review Process Guidelines* (available on the ACCC's website at www.accc.gov.au) the ACCC has established and will maintain an indicative timeline for its assessment of the proposed acquisition. The indicative timeline can change in line with the *Merger Review Process Guidelines*. To keep abreast of possible changes in relation to timing and to find relevant documents, market participants should visit the Mergers Register on the ACCC's website at www.accc.gov.au/mergersregister.
4. A Statement of Issues provides an opportunity for all interested parties (including customers, competitors, shareholders and other stakeholders) to ascertain and consider the primary issues identified by the ACCC. It is also intended to provide the merger parties and other interested parties with the basis for making further submissions should they consider it necessary.

Background

5. The proposed acquisition was publicly announced by J&J on 26 June 2006. On 4 October 2006 J&J provided its submission seeking informal clearance from the ACCC.

The parties

Johnson & Johnson

6. J&J is a global manufacturer of health care products and related services for the consumer, pharmaceutical, and medical devices and diagnostics industries. J&J has more than 230 operating companies in 57 countries and it sells products throughout the world. J&J's portfolio of consumer health care brands include *Band-Aid*, *Tylenol*, *Johnson's Baby*, *Motrin*, *St Joseph aspirin*, *Benecol*, *Splenda*, *Imodium A-D*, *O.B. Tampons*, *Stayfree*, *Mylanta*, *Pepcid AC* and *Monistat*.

Pfizer Inc

7. Pfizer discovers, develops, manufactures and markets pharmaceutical, animal and consumer healthcare products. Pfizer's key consumer healthcare brands include *Benadryl*, *Listerine*, *Nicorette*, *Cortizone*, *Desitin, e.p.t.*, *Lubriderm*, *Neosporin*, *Rolaid*s, *Sudafed* and *Visine*.

Industry background

8. Consumer healthcare markets are impacted in Australia by the regulatory regime governing therapeutic goods. The *Therapeutic Goods Act 1989 (TG Act)*, and accompanying regulations, regulate the supply of medicinal products for therapeutic use in Australia.¹
9. Substances for therapeutic use, that is medicines, are classified under the TG Act as either: registered, listed² or exempt³. Before being supplied in Australia, registered and listed medicines must be entered on the Australian Register of Therapeutic Goods (**ARTG**). The administration of the TG Act and the maintenance of the ARTG are carried out by the Therapeutic Goods Administration (**TGA**). In addition, medicines are classified, on the basis of their active ingredients, in accordance with the schedules to the *Standard for the Uniform Scheduling of Drugs and Poisons (SUSDP)*.
10. Registered medicines are usually included in schedules 2 to schedule 4, which are set out below, of the SUSDP, with progression through these schedules indicating increasingly strict controls.

¹ Medicines, devices or substances for therapeutic use are defined under the TG Act as being those used in or in connection with, relevantly:

(a) preventing, diagnosing, curing or alleviating a disease, ailment, defect or injury; or
(b) influencing, inhibiting or modifying a physiological process (see section 3, TG Act).

² According to J&J, listed medicines are substances which fall within the category of therapeutic goods but are not included in the SUSDP schedules. Such medicines are considered to be safe for general distribution and are able to be sold through retail outlets such as grocery and convenience stores.

³ According to J&J, exempt medicines are substances which are classified as therapeutic goods but which are exempted from entry in the ARTG. Exempt medicines are listed in Schedule 5 of the SUSDP, and as a result of being classified in the 'exempt' category they are still required to comply with the labelling standards which apply to therapeutic goods.

Schedule	Classification	Meaning of classification
Schedule 2	Pharmacy medicine	Substances, the safe use of which may require advice from a pharmacist and which should be available from a pharmacy or, where a pharmacy service is not available, from a licensed person.
Schedule 3	Pharmacist only medicine	Substances, the safe use of which requires professional advice but which should be available to the public from a pharmacist without a prescription.
Schedule 4	Prescription-only medicine	Substances, the use or supply of which should be by or on the order of persons permitted by State or Territory legislation to prescribe and should be available from a pharmacist on prescription.

Source: J&J's submission to the ACCC dated 4 October 2006

11. Non-prescription medicines, including listed and exempt medicines, and products classified in schedules 2 and 3 of the SUSDP, are commonly referred to as over-the-counter (OTC) products.

Market inquiries

12. On 5 October 2006 the ACCC commenced market inquiries regarding the proposed acquisition with a range of interested parties including other manufacturers and distributors of healthcare products, industry associations, and customers for healthcare products, including retail pharmacy chains.

Statement of issues

13. For the purpose of this Statement of Issues, the issues in this matter are divided into three categories: 'issues of concern', 'issues that may raise concerns' and 'issues unlikely to pose concerns'. The ACCC is seeking comments from market participants in relation to all issues.

Issues of concern

Likely effects on competition for supply of worm treatments

14. Both J&J, through its subsidiary Janssen-Cilag Pty Limited (**Janssen**), and Pfizer supply products in Australia for the treatment of worm infestations in humans. Pfizer supplies the brands *Combantrin* and *Combantrin-1* and J&J the *Vermox* brand. According to J&J, *Vermox* and *Combantrin-1* contain the active ingredient of mebendazole, whereas *Combantrin*'s active ingredient is pyrantel. Nonetheless, J&J submits that the active ingredients contained in all of these products fall within schedule 2 of the SUSDP, such that they are all non-prescription medicines which can be obtained OTC from pharmacies.
15. Nonetheless, according to J&J some of the worm treatment products supplied in Australia are prescription-only medicines (schedule 4), including *Anthel*, which is supplied by Alphapharm, and *Eskazole* and *Zentel*, which are supplied by GlaxoSmithKline plc (**GSK**).

16. The ACCC's market inquiries to date suggest that the merged firm would account for approximately 80-85% of the supply of all worm treatments (including prescription-only and OTC treatments) in Australia.
17. J&J submits that in addition to J&J and Pfizer there are twelve other suppliers of worm treatments in Australia, the most significant being GSK, Sigma Company Limited (**Sigma**) including Allied Master Chemists of Australia Limited (**Amcal**), and Alphapharm. The brands of these suppliers include *Eskazole* and *Zentel*, *Deworm* and *Amcal Worm Treatment*, and *Anthel*. It appears that the merged firm's next largest competitor would only have a very small share of the supply of worm treatments in Australia.

Likely effect on competition for supply of anti-diarrhoeals

18. J&J and Pfizer both supply anti-diarrhoeal medications in Australia which relieve the symptoms of diarrhoea.
19. J&J, through Janssen, distribute the *Imodium* range of anti-diarrhoeal products. According to J&J, *Imodium* products contain the anti-diarrhoeal ingredient loperamide HCl, and *Imodium Advanced* also contains the anti-gas ingredient simethicone. If sold in packets of 20 doses or less *Imodium* falls within schedule 2 of the SUSDP and so is a OTC pharmacy medicine (schedule 2).
20. According to J&J, Pfizer distributes two anti-diarrhoeal products, *Lomotil* and *Lofenoxal*. *Lomotil* is intended to be used to treat chronic diarrhoea and its active ingredient is diphenoxylate hydrochloride. When sold in packets of 8 doses or less *Lomotil* falls within schedule 3 of the SUSDP. However, where sold in packets with greater than 8 doses, it is a prescription-only medicine (schedule 4). According to J&J, *Lofenoxal* is a generic product and so is lower priced than *Lomotil*. However, it has the same active ingredient as *Lomotil* and is available for purchase on the same basis.
21. J&J submits that because *Imodium* is most commonly sold as a schedule 2 product and *Lomotil* and *Lofenoxal* as schedule 3 products, its products are less directly competitive with Pfizer's in this product category. Nonetheless, J&J submits that, for the purposes of defining the market, all the anti-diarrhoeal products distributed by J&J and Pfizer are sufficiently substitutable to fall within the same market.
22. The ACCC's market inquiries to date suggest that that the merged firm would have shares of the supply of anti-diarrhoeals in Australia of approximately 55% and 79% in terms of units sold and revenues, respectively.
23. According to J&J, a strong competitor to J&J in this area is Aspen Pharmacare (**Aspen**), through its *Gastrostop* product. J&J also states that other significant competitors for the supply of anti-diarrhoeals in Australia are Hamilton and Sigma, including Amcal. The brands of these suppliers are respectively *Harmonise*, *Diarrhoea Relief* and *Amcal's Anti-diarrhoeal*.

Invitation for further information – worm treatments & anti-diarrhoeals

24. In light of the high level of concentration which is likely to result from the proposed acquisition in relation to the supply of worm treatment and anti-diarrhoeals, and having regard to the ACCC not being satisfied at this stage that existing or potential competitors would be likely to expand supply and provide a sufficient competitive constraint on the merged firm, the ACCC has formed the preliminary view that there is the potential for a significant diminution in competition for the supply of these products.

25. Accordingly, the ACCC is seeking views of interested parties on the extent of substitutability, from a consumer's perspective, of:

- 'OTC' worm treatment products and 'prescription-only' worm treatments; and
- J&J and Pfizer's anti-diarrhoeal products.

Relevant factors in this regard would include the relative prices and functional properties of these different products, including the uses for which doctors and pharmacists recommend the respective products.

26. The ACCC also invites further information from interested parties on the ability and incentive of competitors, or potential competitors, to enter and expand supply of worm treatments and anti-diarrhoeals, with particular reference to the following:

- the obstacles to new competitors entering the supply of, or existing competitors expanding their supply of, worm treatments and anti-diarrhoeals, in light of the incidence of new entry and fluctuations in market shares.

J&J submits that obstacles to expand supply are particularly small in Australian consumer healthcare markets, and that this is illustrated by the regular product entry and large fluctuations in market shares of suppliers of worm treatments and anti-diarrhoeals. J&J asserts that the entry of Aspen to the Australian healthcare industry in 2002, and particularly the rapid growth of Aspen's *Gastrostop* brand, is an example of the ease of this entry and expansion;

- the ease with which, in terms of cost and time, worm treatment and anti-diarrhoeal products can be listed or registered with the TGA, as required by the TG Act and regulations, before they can be sold in Australia.

J&J submits that, particularly with respect to OTC products, the registration and listing procedures are not particularly onerous;

- the extent to which brand awareness acts as an obstacle to entry and expansion to the supply of worm treatments and anti-diarrhoeals. In particular, interested parties are invited to provide further information in relation to:

- The role that the recommendations of physicians and pharmacists has in influencing consumer purchasing decisions for these products.

While J&J recognises that brand recognition is a factor influencing consumer purchasing decisions for healthcare products, it submits that ultimately the recommendation of a physician or pharmacist plays an important role.

- The total marketing and promotion spend by the owners of worm treatment and anti-diarrhoeal brands that is directed at health professionals rather than end-consumers;
- the extent to which patent protection and product development act as obstacles to entry and expansion to the supply of worm treatments and anti-diarrhoeals.

J&J submits that it is not aware of any patents used by it (or its subsidiaries) and Pfizer for the manufacture of products in the anti-diarrhoeal or worm treatment categories. J&J also submits that most consumer healthcare products do not require significant investment in research and development for their development;

- the extent to which the trend toward retail pharmacies participating in banner, franchise and buying group arrangements would impose a measure of countervailing power on the merged firm.

J&J notes that approximately 36% of Australian pharmacies are members of banner groups. J&J submits that due to the significant changes which have been occurring in the pharmacy retail channel, pressure is mounting for further increases in the proportion of pharmacies participating in banner group, franchise and buying group arrangements; and

- the ability and incentive of retail pharmacies to expand their supply of generic brands, including the extent of consumer acceptance of generic brands.

J&J states that the current branding of a private label product is linked to the chain of retail pharmacies to which it belongs, the most prominent of such ranges being: *Amcal's Own*, *Guardian*, *Soul Pattinson*, *Terry White Chemists* and *Chemists' Own*. The brands of these suppliers in the worm treatments and anti-diarrhoeal categories are *Amcal Worm Treatment* and *Amcal Anti-diarrhoea*, *Guardian Worm Treatment* and *Guardian Diarrhoea Relief*, *Soul Pattinson Diarrhoea*, and *Terry White Worming and Terry White Diarrhoea Relief*. In addition, J&J states that Australian consumer acceptance of private label brands was the highest in the Asia Pacific region.⁴

⁴ AC Neilson Online Consumer Opinion Survey 2005.

Likely effect on competition for supply of nicotine replacement therapy products

27. Nicotine replacement therapy (NRT) products are designed to assist the treatment of cigarette addiction.
28. Alza Corporation (**Alza**), a wholly owned subsidiary of J&J, develops and manufactures transdermal drug delivery mechanisms. GSK obtains supply of NRT patches pursuant to an exclusive distribution agreement between Alza and GSK for the supply of nicotine patches in Australia. The distribution agreement was executed in 1997 and it commenced a long-term distribution relationship between Alza and GSK. The distribution agreement expires at the end of [redacted per request of GSK], but could be extended by mutual written agreement of the parties.
29. GSK markets and distributes NRT patches in Australia under the *Nicabate CQ* brand. The *Nicabate CQ* brand also includes other NRT products such as gum and lozenges.
30. Pfizer manufactures and distributes various NRT products in Australia, including patches, gum, sublingual tablets, nasal sprays and inhalers, under its *Nicorette* brand.
31. Based on market inquiries to date, the ACCC has preliminary concerns that there is likely to be a substantial lessening of competition in the NRT area post-acquisition, as J&J may have significant influence in relation to the NRT patch products of the two leading Australian brands, *Nicabate CQ* and *Nicorette*. It appears that *Nicabate CQ* and *Nicorette* would together account for approximately 65% of total Australian NRT patch sales.
32. Market inquiries to date suggest that as a result of the proposed acquisition:
 - J&J would have access to competitively sensitive information relating to a major competing NRT product;
 - J&J would have the ability and incentive to reduce the competitiveness of a major NRT competing product; and
 - there may not be an alternative source of competitive supply of NRT patches.
33. However, J&J submits that its ability and incentive to foreclose GSK to the detriment of competition would not be increased by the proposed acquisition because:
 - post-acquisition J&J would face a constraint from other suppliers of NRT products;
 - J&J (Alza) cannot rescind its obligation to supply GSK;
 - Alza could not raise prices supra-competitively;

- GSK is not dependent upon J&J (Alza) for supply of NRT patches. J&J submits that there are a number of other companies which supply nicotine patches which GSK could turn to, including Lohmann Therapy Systems, Aveva DDS, Watson Pharmaceuticals and Samyang Corporation. Further, J&J submits that other manufacturers of transdermal patches for other drug deliveries would be potential sources of supply, including Mylan Technologies Inc,⁵ Noven Pharmaceuticals Inc⁶ and 3M⁷; and
- J&J (Alza) and GSK's arrangements only relate to NRT patch products, so J&J would continue to face competition from other NRT products post-acquisition.

Invitation for further information – Nicotine Replacement Therapy

34. In light of the information set out above, the ACCC has formed the preliminary view that there is likely to be a substantial lessening of competition for the supply of NRT products if the proposed acquisition proceeds. Accordingly, the ACCC invites further information from interested parties regarding:

- the substitutability from a consumer's perspective of nicotine patch, gum, lozenge and inhaler products;
- whether other current global manufacturers of nicotine patches represent a viable source of supply for GSK in relation to the Australian nicotine patch market;
- whether there are significant qualitative differences between the products GSK acquires from Alza and the other patch products which are available globally; and
- the ability of manufacturers of transdermal patches to supply nicotine patches. In particular, please provide views in relation to the ease with which (taking account of cost and time considerations) the supplier of a non-nicotine patch could adapt to supplying nicotine patches.

Issues that may raise concerns

Likely effect on competition for supply of anti-allergics

35. According to J&J, in relation to medications available to treat allergic reactions containing some form of antihistamine, Pfizer sells a *systemic* anti-allergy medicine (which affects the entire body as it is taken orally) while J&J sells a *topical* anti-allergy medicine (which is applied externally and has a localised effect). Pfizer supplies the *Zyrtec* brand which is available in tablet forms, as oral drops, and as a solution. J&J supplies the *Livostin* brand in the form of a nasal spray and eye drops. Both the active ingredients in *Livostin* and *Zyrtec* are a form of antihistamine and the products are available OTC.

⁵ According to J&J, Mylan Technologies Inc is the largest manufacturer of transdermal generics in the United States and it has the required technology to manufacture nicotine patches.

⁶ According to J&J, Noven has a nicotine patch in development.

⁷ J&J submits that 3M manufactures a range of transdermal patches.

36. J&J submits that systemic and topical anti-allergy medicines sometimes have complementary uses, but are not substitutable from a consumer's perspective as they have different modes of delivery, pharmacodynamics and uses. In particular, according to J&J, systemic anti-allergy medicines generally relieve a broader range of allergic symptoms and have longer effects, as well as more serious side effects, than topical anti-allergy medicines.
37. Pfizer also sells a range of ocular decongestants through its *Visine* brand, the active ingredient of which is tetrazyoline or tetrahydrozoline. J&J submits that ocular decongestants relieve occasional redness, irritation and dryness of eyes. According to J&J, ocular decongestants such as Pfizer's *Visine* eye drops and ocular anti-allergics such as J&J's *Livostin* product are also not substitutable from a consumer's perspective as they have different purposes and different methods of action.
38. In addition, according to J&J, the nasal spray product it supplies is not substitutable with the eye drops supplied by Pfizer.
39. Accordingly, J&J submits that the only competitive overlap between the merger parties in the anti-allergy category is in the area of ocular antihistamines. That is, between Pfizer's *Visine Allergy* eye drops and J&J's *Livostin* eye drops.
40. The ACCC's market inquiries suggest that the merged firm would account for approximately 30% of the supply of ocular antihistamine products in Australia. The merged firm would face competition from Alcon Laboratories and Novartis International AG.

Invitation for further information – anti-allergics (ocular antihistamines)

41. The ACCC considers that the competition effects arising from the proposed acquisition in relation to the supply of anti-allergics requires further analysis, and is therefore unable to form a preliminary view regarding this product area at this stage. Accordingly, the ACCC requests further information and views on:
 - the substitutability from a consumer's perspective of systemic and topical anti-allergy medicines;
 - the substitutability from a consumer's perspective of ocular decongestants and ocular anti-allergics; and
 - the ability and incentive of competitors, or potential competitors, to enter and expand supply of ocular antihistamines in Australia.

Possible competition concern arising from merged firm's wider product portfolio

42. The ACCC will examine how the merged firm's ability to offer retailers a wider portfolio of products, that may be regarded as complements by pharmacists and/or supermarkets, would affect competition. The ACCC will examine whether the merged firm would have the ability and incentive to leverage any

market strength in one or more products to increase sales of a complementary⁸ product/s with the effect of significantly foreclosing rivals' access to shelf space, thereby raising their costs to the detriment of competition. The factors that the ACCC will take into account, and on which the ACCC invites interested parties to provide information, include:

- the extent of retailers' incentives (including transaction cost savings, convenience and/or discounts/rebates) to accept a bundle of products;
 - any history of bundling or tying by the merger parties and their competitors;
 - the ability of retailers to bypass the merged firm's tying of the supply of one product to another, by sourcing from an alternative supplier/s; and
 - the ability of current or new competitors to offer a comparable bundle of products, having regard to consumers' brand loyalty, intellectual property restrictions, and product development delays.
43. J&J's submits that there is no prospect of the proposed acquisition giving rise to any portfolio or conglomerate issues because:
- there will be little, if any, complementary products in the merged firm's portfolio;
 - no products in the merged firm's product portfolio have sufficient weight to enable them to be used as a lever through bundling;
 - the absence of retailer incentives to accept bundled product offerings;
 - retailers in both the pharmacy and grocery channel possess sufficient countervailing buying power to defeat any unwanted product bundles;
 - competitors are capable of offering bundled product promotions that are equally, if not more, attractive than those able to be offered by J&J; and
 - neither merger party has any history of implementing forced or otherwise anti-competitive bundling arrangements.

Issues unlikely to pose concerns

44. At this stage, the ACCC considers that the proposed acquisition is unlikely to pose substantial competition concerns in relation to the supply of:

⁸ Essentially, complementarity occurs where there is significant commonality of customers for products and the strength of demand for one product is positively correlated with the strength of demand for another. This may occur for technical reasons (such as the products having to be consumed together). Complementarity may also arise for commercial reasons, such as where products form part of a range of products that distributors/retailers need to carry to meet end-consumer demand. In the present case, it is plausible that a number of consumer healthcare products relevant to the proposed acquisition would form a range of consumer healthcare products which pharmacists and/or grocery retailers would need to stock to meet end-consumer demand.

- nappy rash treatments;
 - codeine phosphate products;
 - thrush treatments;
 - cold and flu treatments; and
 - analgesics.
45. J&J submits that while there is a horizontal overlap in Australia between J&J and Pfizer's supply of nappy rash treatment products, the supply of these products is not highly concentrated. It appears the merged firm would face strong competition from Bayer, Heinz and Church & Dwight.
 46. J&J, through its subsidiaries Tasmanian Alkaloids Pty Limited (**TasAlk**) and Noramco Inc (**Noramco**), supplies the input codeine phosphate which is an active ingredient in several products marketed by Pfizer in Australia. However, according to J&J, Pfizer is a very small consumer of codeine phosphate, and as such it does not appear that J&J's ownership of TasAlk or Noramco gives rise to competition concerns.
 47. Nonetheless, the ACCC will accept further submissions from industry participants and will further consider these product areas if it considers that such an assessment is warranted.

ACCC's future steps

48. The ACCC will finalise and publicly announce its view on this matter after it considers market responses invited by this Statement of Issues.
49. The ACCC now seeks submissions from industry participants on each of the issues identified in this Statement of Issues and on any other issue that may be relevant to the ACCC's assessment of this matter.
50. Submissions are to be received by the ACCC no later than 11 December 2006. The ACCC will consider the submissions received from the market and the merger parties in light of the issues that are identified above and will, in conjunction with information and submissions already provided by the merger parties, come to a final view as to the appropriate course of action to take to resolve any competition concerns that remain.
51. A Public Competition Assessment for the purpose of explaining the ACCC's final view on the proposed acquisition may be published following the ACCC's public announcement of its view.